ANGELA M. TORRES, LCSW

TECHNICAL QUALIFICATION NARRATIVES

Challenge • Context • Action • Result (CCAR) | Prepared for agency interviews

How to use: OPM's 2025 SES guidance prohibits TQ narratives in the application; TQs must be shown in the two-page resume and are probed in the agency interview. Replace these TQ titles with the exact wording in the vacancy announcement, then use the narratives to prepare and reduce each one to key points.

TQ 1 HEALTHCARE LEADERSHIP

Key elements: *Clinical quality oversight* • *Patient safety* • *Interdisciplinary teams* • *National program design*

Creating a behavioral health investigation track adopted across VHA (2023–2025)

CHALLENGE

As Director of Western Region Investigations, I saw that our investigations of suicide-related events, residential treatment, and homeless programs were missing root causes. My challenge was to build an investigation method that fit the realities of behavioral health care.

CONTEXT

Behavioral health cases made up a growing share of the disclosures and adverse events in my 52-facility region, but they were reviewed with general medical methods by teams that often lacked mental health expertise. Facilities questioned findings that did not reflect how behavioral health care actually works, and the same problems kept recurring.

ACTION

- **Designed the method:** I wrote specialized protocols for suicide prevention, residential treatment, and homeless programs, built on root cause analysis and my own clinical experience as a suicide prevention coordinator and social work chief.
- **Built the right teams:** I assembled interdisciplinary teams of psychiatrists, psychiatric nurses, psychologists, and social workers, and trained reviewers in behavioral health standards and safety planning.
- **Proved it worked:** I piloted the track in the western region, tracked whether facilities agreed with and carried out its recommendations, and refined the protocols based on what we learned.
- **Scaled it:** I briefed the Medical Inspector and national leaders on the results and helped other regions adopt the method.

RESULT

- The track became the national model for behavioral health investigations in the Office of the Medical Inspector.
- It contributed to 140+ regional investigations and 600+ recommendations with 94% concurrence.
- Its findings laid the groundwork for the national effort that closed suicide-risk screening gaps at 40+ sites.

TQ 2 EXPERT KNOWLEDGE OF BEHAVIORAL HEALTH SERVICE DELIVERY AND SUICIDE PREVENTION

Key elements: *Clinical expertise* • *Access to care* • *Service-line management* • *Suicide prevention*

Cutting mental health wait times from 21 days to 9 at VA Salt Lake City (2019)

CHALLENGE

During a detail as Acting Associate Chief of Behavioral Health, I was asked to cut mental health wait times for Veterans at a major medical center and its clinics, while leading a 300-person service line.

CONTEXT

Veterans were waiting an average of 21 days for mental health appointments, a serious risk for those in crisis. Clinician vacancies, inconsistent scheduling across clinics, and limited telehealth meant capacity was being lost. I had five months to make lasting change.

ACTION

- **Triage first:** I set up same-day access for Veterans with urgent needs and standard risk screening at first contact, drawing on my years building high-risk tracking as a suicide prevention coordinator.
- **Recovered capacity:** I standardized scheduling across clinics, moved clinician time to the busiest sites, and added group therapy for common conditions.
- **Expanded access:** I increased telehealth appointments so Veterans in rural areas could be seen without traveling.
- **Measured weekly:** I reviewed wait-time data with clinic leaders every week and adjusted staffing where delays persisted.

RESULT

- Cut average mental health wait times from 21 days to 9 within the five-month detail.
- Ensured Veterans with urgent needs were seen the same day.
- Left a scheduling and triage model the service line kept after the detail ended.